

WHY ARE ER VISITS EXPLODING ?

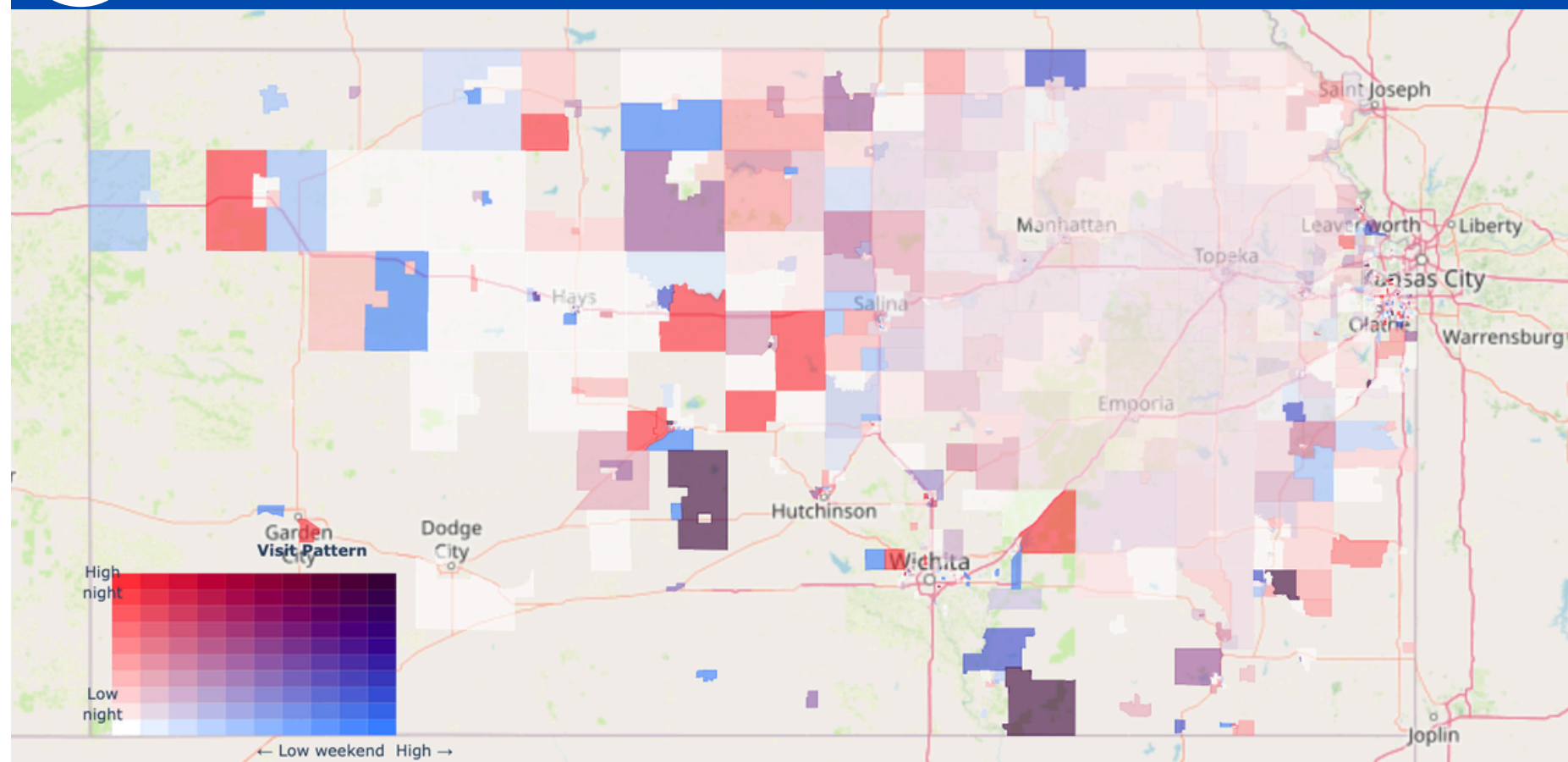
WHAT PATIENT AND SYSTEM FACTORS
ARE LEADING TO OVERWHELMING ER
USAGE?

BY MESSAGE CENTER (D13)

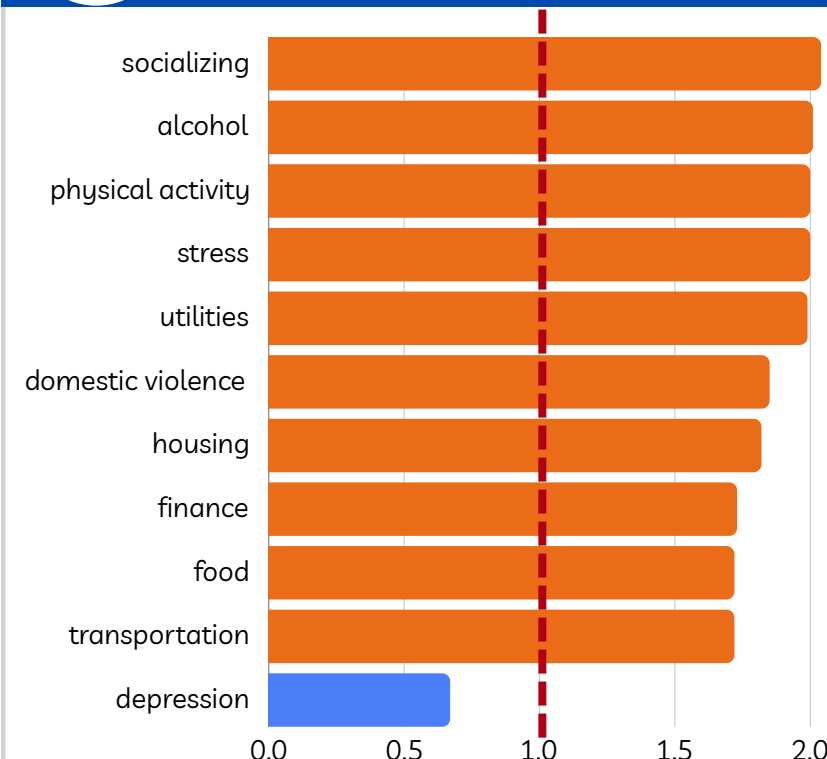
Aaron, Tim, Maggie, Danny, Freya



D1 No non-ER choices during close hours

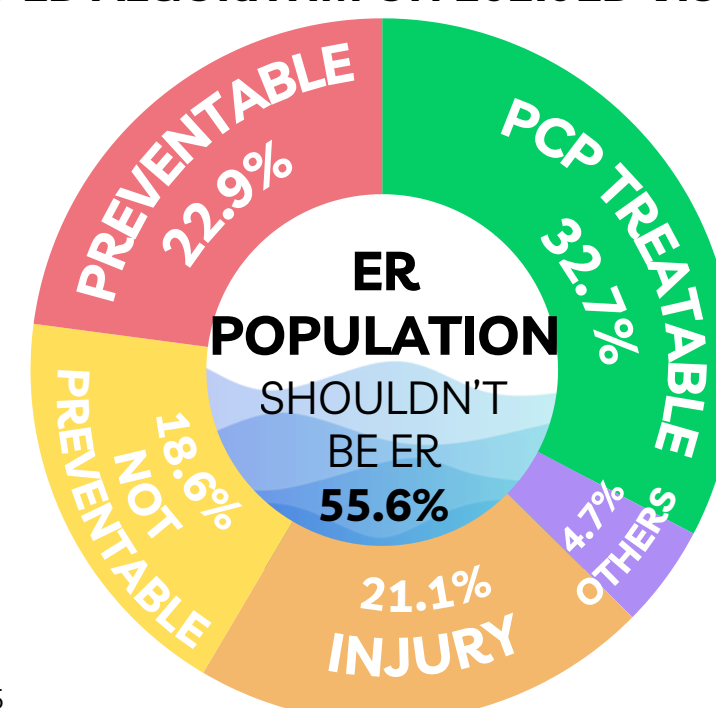


D2 ER Defaulter & SDOH Returners



SDOH: “Random Place, GO! 🏃”
ER: “Fine, we’ll take you 😞”

NYU ED ALGORITHM ON 202K ED VISITS



ER Returners: “Random Sickness: GO!!”
ER: “Pls don’t, go elsewhere”

“

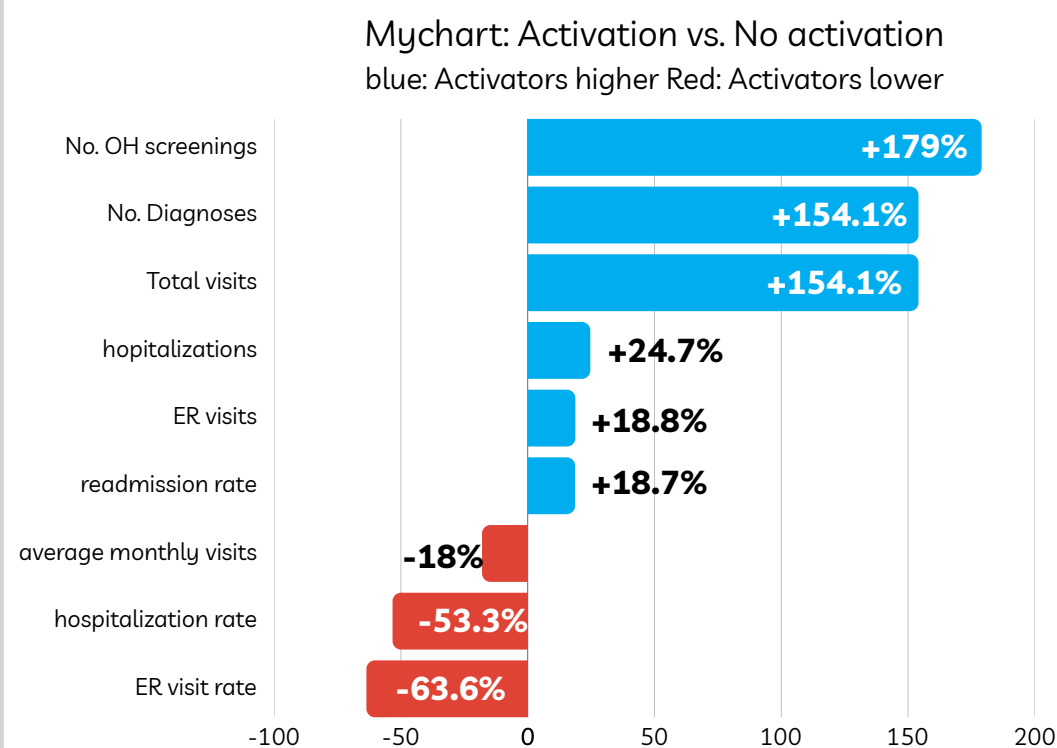
What patient and system factors drive avoidable ED use, and to what extent do clinic hour gaps, lack of a primary care relationship, and social resource scarcity each independently predict ED visit rates?



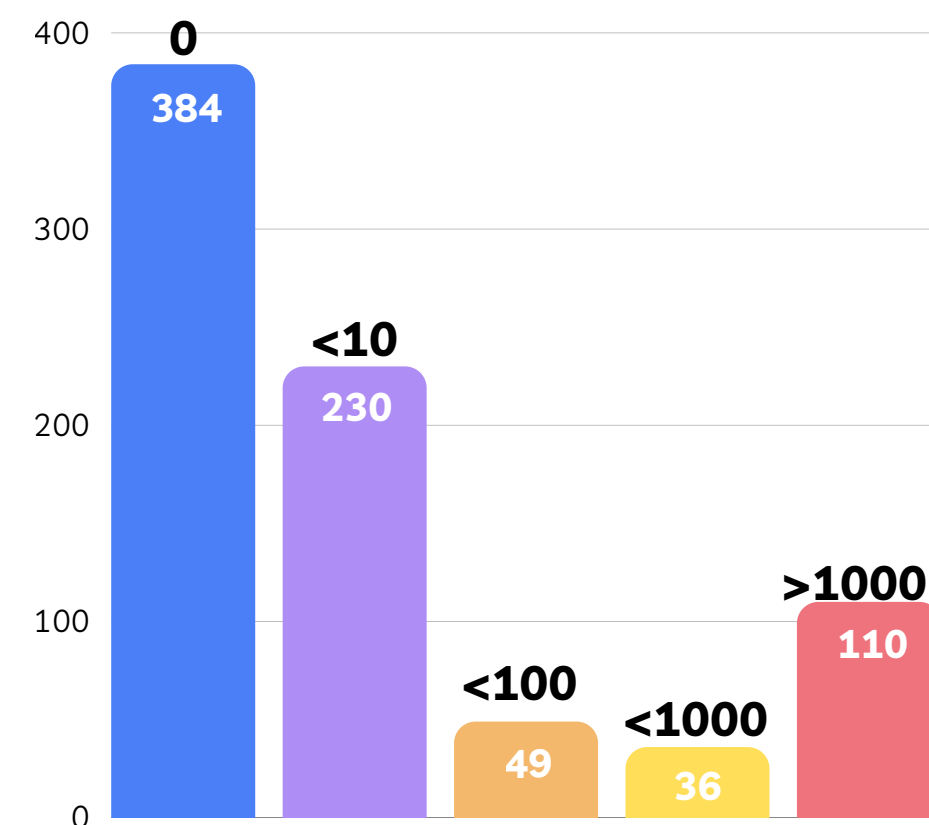
Using 947,685 patients, 7.7M encounters, and the NYU ED Algorithm on 202K ED visits, **we find that 55.6% of ED surge are broadly avoidable.** And 110 PCPs are carrying **275,440** patients per year.

Scan the QR code for the full report!

D3 Mychart: Use vs No use

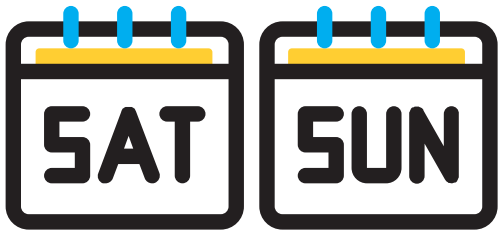


S1 Poor PCP Workload Distribution



Demand side solution: diverge patients

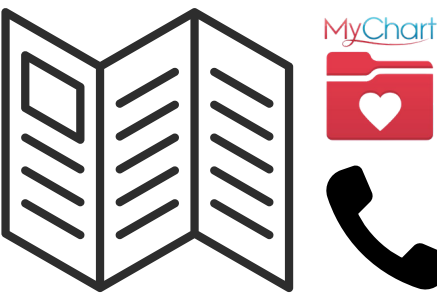
Extra Clinic Hours



Upgrade Triage



Resource Referral

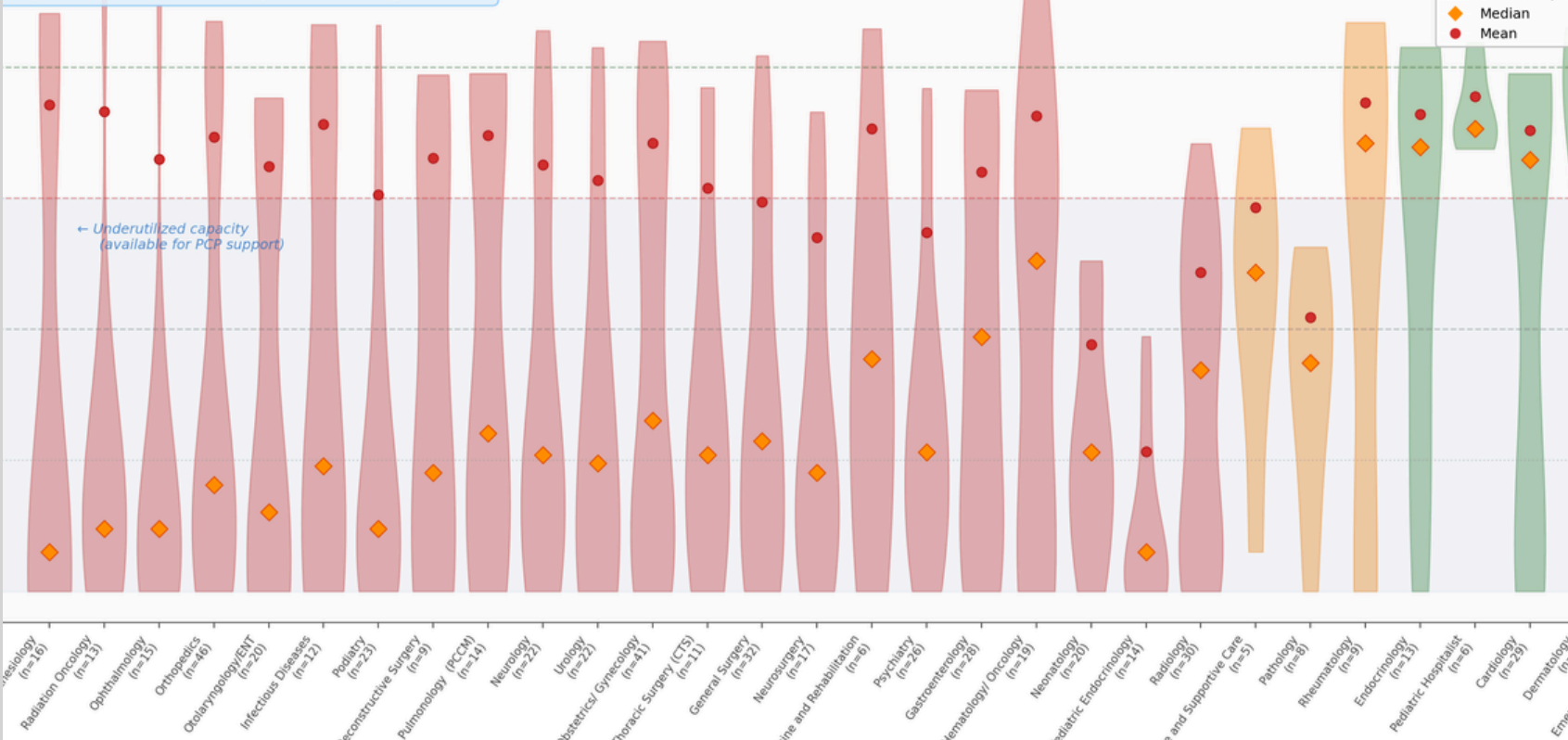


Projected Effect of Solution Bundle

intervention	p value	r	effect metric	Projected ED Reduction
Extra Clinic Hours	2.45E-5	-0.0107	Cramér's V = 0.0107	3.53%
Upgrade Triange	0	0.3613	AUC = 0.6806	17.06%
MyChart/SDOH Brochure	2.47E-163	-0.1885	Cohen's d = -0.363	0.26%
Specialist Reallocation	0	-0.173	OLS slope = -0.135	0%
Overall	0	0.2659	R² = 0.0707	22.06%

Supply side solution: idle specialist!

611 active specialists across 32 specialties
insight: In most specialties, median < mean → majority have low activity, carry extreme load → available capacity for PCP rebalancing



315 specialists who could serve in PCP roles are underworking:
since specialists share the same general preliminary medical training that PCP roles rely on, which is underutilized when PCP is lacking and specialists are idling

Conclusion

Avoidable ED visits are driven not by patient choices but by access gaps: limited clinic hours, absent primary care relationships, and social resource scarcity. Closing these gaps through extended hours, enhanced triage, community navigation, and specialist rebalancing can meaningfully reduce ED burden within existing resources.